

Ponatinib / Asciminib nella LMC – Newsletter interna (ultimi 15 giorni)

Generata il 16/07/2026

Generata il: 2026-07-16

Newsletter interna – sintesi operativa

Messaggi chiave (da leggere in 60 secondi):

- Nel periodo emergono soprattutto temi su **Safety e comorbidità** (n=5).
- Secondo filone: **Sequencing / linee di terapia** (n=4).
- Terzo filone: **TFR / stop terapia** (n=3).

Cosa cambia per noi (azioni pratiche):

- Rivedere/rafforzare **monitoraggio cardiovascolare e interazioni** (es. anticoagulanti) nei pazienti in TKI.
- Valutare criteri interni per **consolidamento / de-escalation / TFR** nei pazienti in deep response.
- Verificare percorso di **test mutazionale** e criteri di switch nelle resistenze (incl. T315I).

Safety radar (segnali/interazioni da monitorare):

- Monitorare evoluzione su **TFR / stop terapia** (nuovi segnali/dati).
- Monitorare evoluzione su **Safety e comorbidità** (nuovi segnali/dati).
- Monitorare evoluzione su **Resistenza / T315I / mutazioni** (nuovi segnali/dati).

Top 5 da leggere subito (ordinati per impatto)

1) Asciminib as first-line therapy in chronic myeloid leukemia: a systematic review.

Linee guida / consenso

Blood research

Score: 18

Focus clinico: TFR / stop terapia, Safety e comorbidità, Sequencing / linee di terapia, Sintesi (review/linee guida)

PMID: 42458108 | 10.1007/s44313-026-00153-2

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42458108/>

- **Tipo di studio/popolazione:** This systematic review evaluated the efficacy and safety of asciminib as first-line therapy in patients with newly diagnosed chronic-phase CML.
- **Risultato chiave:** Eligible studies included randomized controlled trials, prospective cohort studies, and compassionate use programs evaluating asciminib as first-line therapy in adult patients with chronic-phase CML.
- **Rilevanza clinica:** Treatment discontinuation due to adverse events was lower with asciminib (5.0%) than with second-generation TKIs (12.7%).

2) Therapeutic drug monitoring guides personalized ponatinib dose reduction for chronic myeloid leukemia.

Trial clinico

International journal of hematology

Score: 16

Focus clinico: Dose strategy / consolidamento, Safety e comorbidità, Sequencing / linee di terapia

PMID: 42449088 | 10.1007/s12185-026-04254-7

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42449088/>

- **Tipo di studio/popolazione:** Although the phase II OPTIC trial showed that reducing the dose to 15 mg daily (QD) after achieving a response provided an optimal benefit-risk profile, this has not been adequately verified using plasma drug concentrations data.
- **Risultato chiave:** A 66-year-old woman with chronic-phase chronic myeloid leukemia (CP-CML) was switched to ponatinib because of nilotinib-induced grade 3 thrombocytopenia.
- **Rilevanza clinica:** A major molecular response (MMR) was achieved 30.6 months after starting ponatinib.

3) Ponatinib in CML and Ph+ ALL: balancing high efficacy with vascular safety.

Real-world / osservazionale

Postgraduate medicine

Score: 15

Focus clinico: Resistenza / T315I / mutazioni, Safety e comorbidità, Real-world (utilità pratica)

PMID: 42436599 | 10.1080/00325481.2026.2691687

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42436599/>

- **Tipo di studio/popolazione:** Ponatinib, a third-generation TKI, uniquely retains activity against the T315I 'gatekeeper' mutation and most other BCR:ABL1 variants, producing deep cytogenetic and molecular responses in resistant CML and improving survival in Ph+ ALL.
- **Risultato chiave:** Across the PACE and OPTIC programs, frontline Ph+ ALL combinations (hyper-CVAD- and blinatumomab-based), and real-world cohorts, ponatinib demonstrates robust antileukemic efficacy in diverse settings and after multiple prior TKIs.
- **Rilevanza clinica:** Clinically, response-based dosing-such as step-down from 45→30→15 mg in CML or 30→15 mg in Ph+ ALL upon molecular milestones-preserves efficacy while reducing exposure-related AOE risk, though events are not eliminated.

4) Reattempt of tyrosine kinase inhibitor discontinuation after maintenance therapy with ponatinib in patients with chronic myeloid leukaemia in the chronic phase: Result of the Japan Adult Leukemia Study Group RE-STOP219 study.

Altro

British journal of haematology

Score: 14

Focus clinico: TFR / stop terapia, Sequencing / linee di terapia

PMID: 42422985 | 10.1111/bjh.70678

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42422985/>

- **Tipo di studio/popolazione:** Patients who had relapsed after prior tyrosine kinase inhibitor (TKI) discontinuation and subsequently regained molecular response 4.5 (MR4.5; defined as a BCR-ABL1 transcript level $\leq 0.0032\%$ on the International Scale [IS]) were enrolled, switched to ponatinib (15 mg/day) for 12 months and those who maintained MR4.5 discontinued TKI therapy.
- **Risultato chiave:** Of 50 enrolled patients, 49 received ponatinib and 41 proceeded to TKI discontinuation.
- **Rilevanza clinica:** In conclusion, ponatinib maintenance followed by a second attempt at TKI discontinuation may facilitate successful TFR in selected CML patients.

5) Favorable tolerability of asciminib *versus* ATP-competitive tyrosine kinase inhibitors in the ASC4FIRST study of newly diagnosed patients with chronic-phase chronic myeloid leukemia.

Trial clinico

Haematologica

Score: 12

Focus clinico: TFR / stop terapia, Dose strategy / consolidamento, Safety e comorbidità

PMID: 42389818 | 10.3324/haematol.2025.300145

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42389818/>

- **Tipo di studio/popolazione:** Many patients with chronic myeloid leukemia (CML) treated with adenosine triphosphate (ATP)-competitive tyrosine kinase inhibitors (TKIs) experience persistent adverse events (AEs) that negatively impact daily living and the ability to remain on treatment.
- **Risultato chiave:** The phase 3 randomized ASC4FIRST trial established the overall favorable safety profile of asciminib in patients with newly diagnosed CML in chronic phase (CP).
- **Rilevanza clinica:** The safety and tolerability of asciminib observed in the ASC4FIRST trial demonstrate asciminib's excellent benefit-risk profile as a frontline therapy for a broad range of patients with newly diagnosed CML-CP.

Evidenze per domande cliniche

Safety e comorbidità (5)

Score: 0

Mini-riassunto (da abstract, estrattivo):

Asciminib as first-line therapy in chronic myeloid leukemia: a systematic review.

Linee guida / consenso

Blood research

Score: 18

PMID: 42458108 | 10.1007/s44313-026-00153-2

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42458108/>

Open Access (Europe PMC/PMC): non rilevato

- **Tipo di studio/popolazione:** This systematic review evaluated the efficacy and safety of asciminib as first-line therapy in patients with newly diagnosed chronic-phase CML.
- **Risultato chiave:** Eligible studies included randomized controlled trials, prospective cohort studies, and compassionate use programs evaluating asciminib as first-line therapy in adult patients with chronic-phase CML.
- **Rilevanza clinica:** Treatment discontinuation due to adverse events was lower with asciminib (5.0%) than with second-generation TKIs (12.7%).

Mini-riassunto (da abstract, estrattivo):

Therapeutic drug monitoring guides personalized ponatinib dose reduction for chronic myeloid leukemia.

Trial clinico

International journal of hematology

Score: 16

PMID: 42449088 | 10.1007/s12185-026-04254-7

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42449088/>

Open Access (Europe PMC/PMC): non rilevato

- **Tipo di studio/popolazione:** Although the phase II OPTIC trial showed that reducing the dose to 15 mg daily (QD) after achieving a response provided an optimal benefit-risk profile, this has not been adequately verified using plasma drug concentrations data.
- **Risultato chiave:** A 66-year-old woman with chronic-phase chronic myeloid leukemia (CP-CML) was switched to ponatinib because of nilotinib-induced grade 3 thrombocytopenia.
- **Rilevanza clinica:** A major molecular response (MMR) was achieved 30.6 months after starting ponatinib.

Mini-riassunto (da abstract, estrattivo):

Ponatinib in CML and Ph+ ALL: balancing high efficacy with vascular safety.

Real-world / osservazionale

Postgraduate medicine

Score: 15

PMID: 42436599 | 10.1080/00325481.2026.2691687

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42436599/>

Open Access (Europe PMC/PMC): non rilevato

- **Tipo di studio/popolazione:** Ponatinib, a third-generation TKI, uniquely retains activity against the T315I 'gatekeeper' mutation and most other BCR:ABL1 variants, producing deep cytogenetic and molecular responses in resistant CML and improving survival in Ph+ ALL.
- **Risultato chiave:** Across the PACE and OPTIC programs, frontline Ph+ ALL combinations (hyper-CVAD- and blinatumomab-based), and real-world cohorts, ponatinib demonstrates robust antileukemic efficacy in diverse settings and after multiple prior TKIs.
- **Rilevanza clinica:** Clinically, response-based dosing-such as step-down from 45→30→15 mg in CML or 30→15 mg in Ph+ ALL upon molecular milestones-preserves efficacy while reducing exposure-related AOE risk, though events are not eliminated.

Mini-riassunto (da abstract, estrattivo):

Favorable tolerability of asciminib *versus* ATP-competitive tyrosine kinase inhibitors in the ASC4FIRST study of newly diagnosed patients with chronic-phase chronic myeloid leukemia.

Trial clinico

Haematologica

Score: 12

PMID: 42389818 | 10.3324/haematol.2025.300145

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42389818/>

Open Access (Europe PMC/PMC): non rilevato

- **Tipo di studio/popolazione:** Many patients with chronic myeloid leukemia (CML) treated with adenosine triphosphate (ATP)-competitive tyrosine kinase inhibitors (TKIs) experience persistent adverse events (AEs) that negatively impact daily living and the ability to remain on treatment.
- **Risultato chiave:** The phase 3 randomized ASC4FIRST trial established the overall favorable safety profile of asciminib in patients with newly diagnosed CML in chronic phase (CP).
- **Rilevanza clinica:** The safety and tolerability of asciminib observed in the ASC4FIRST trial demonstrate asciminib's excellent benefit-risk profile as a frontline therapy for a broad range of patients with newly diagnosed CML-CP.

Mini-riassunto (da abstract, estrattivo):

Pharmacokinetics of asciminib in a patient with chronic myeloid leukemia undergoing maintenance hemodialysis.

Altro

Cancer chemotherapy and pharmacology

Score: 9

PMID: 42412118 | 10.1007/s00280-026-04919-4

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42412118/>

Open Access (Europe PMC/PMC): non rilevato

- **Tipo di studio/popolazione:** A 72-year-old man with chronic phase chronic myeloid leukemia undergoing maintenance hemodialysis for chronic renal failure was treated with ASC.
- **Risultato chiave:** A major molecular response was achieved within 3 months without adverse events.
- **Rilevanza clinica:** ASC demonstrated negligible dialyzability, likely because of its high plasma protein binding (97.3%).

Sequencing / linee di terapia (4)

Score: 0

Mini-riassunto (da abstract, estrattivo):

Asciminib as first-line therapy in chronic myeloid leukemia: a systematic review.

Linee guida / consenso

Blood research

Score: 18

PMID: 42458108 | 10.1007/s44313-026-00153-2

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42458108/>

Open Access (Europe PMC/PMC): non rilevato

- **Tipo di studio/popolazione:** This systematic review evaluated the efficacy and safety of asciminib as first-line therapy in patients with newly diagnosed chronic-phase CML.
- **Risultato chiave:** Eligible studies included randomized controlled trials, prospective cohort studies, and compassionate use programs evaluating asciminib as first-line therapy in adult patients with chronic-phase CML.
- **Rilevanza clinica:** Treatment discontinuation due to adverse events was lower with asciminib (5.0%) than with second-generation TKIs (12.7%).

Mini-riassunto (da abstract, estrattivo):

Therapeutic drug monitoring guides personalized ponatinib dose reduction for chronic myeloid leukemia.

Trial clinico

International journal of hematology

Score: **16**

PMID: 42449088 | 10.1007/s12185-026-04254-7

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42449088/>

Open Access (Europe PMC/PMC): non rilevato

- **Tipo di studio/popolazione:** Although the phase II OPTIC trial showed that reducing the dose to 15 mg daily (QD) after achieving a response provided an optimal benefit-risk profile, this has not been adequately verified using plasma drug concentrations data.
- **Risultato chiave:** A 66-year-old woman with chronic-phase chronic myeloid leukemia (CP-CML) was switched to ponatinib because of nilotinib-induced grade 3 thrombocytopenia.
- **Rilevanza clinica:** A major molecular response (MMR) was achieved 30.6 months after starting ponatinib.

Mini-riassunto (da abstract, estrattivo):

Reattempt of tyrosine kinase inhibitor discontinuation after maintenance therapy with ponatinib in patients with chronic myeloid leukaemia in the chronic phase: Result of the Japan Adult Leukemia Study Group RE-STOP219 study.

Altro

British journal of haematology

Score: **14**

PMID: 42422985 | 10.1111/bjh.70678

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42422985/>

Open Access (Europe PMC/PMC): non rilevato

- **Tipo di studio/popolazione:** Patients who had relapsed after prior tyrosine kinase inhibitor (TKI) discontinuation and subsequently regained molecular response 4.5 (MR4.5; defined as a BCR-ABL1 transcript level $\leq 0.0032\%$ on the International Scale [IS]) were enrolled, switched to ponatinib (15 mg/day) for 12 months and those who maintained MR4.5 discontinued TKI therapy.
- **Risultato chiave:** Of 50 enrolled patients, 49 received ponatinib and 41 proceeded to TKI discontinuation.
- **Rilevanza clinica:** In conclusion, ponatinib maintenance followed by a second attempt at TKI discontinuation may facilitate successful TFR in selected CML patients.

Mini-riassunto (da abstract, estrattivo):

Financial Impact of Treatment Choice in Chronic Myeloid Leukemia: A Comparison of Later Generation TKIs versus Imatinib from Patient and Payer Perspectives.

Review ClinicoEconomics and outcomes research : CEOR Score: 5

PMID: 42445105 | 10.2147/CEOR.S626752

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42445105/>

Open Access (Europe PMC/PMC): non rilevato

- **Tipo di studio/popolazione:** The financial impact of later-generation TKIs (asciminib, bosutinib, dasatinib, nilotinib, and ponatinib) and imatinib was measured from patients' and payers' perspectives before (2024) and after the implementation of Inflation Reduction Act (2025), respectively.
- **Risultato chiave:** An all-payer blended US health plan with 1 million members could expect to have 200 patients with Philadelphia chromosome-positive CML eligible for TKI therapy.
- **Rilevanza clinica:** Both imatinib and later-generation TKI costs were projected to exceed annual OOP caps in 2025, likely resulting in minimal change in insured CML patients' OOP costs.

TFR / stop terapia (3)

Score: 0

Mini-riassunto (da abstract, estrattivo):

Asciminib as first-line therapy in chronic myeloid leukemia: a systematic review.

Linee guida / consenso Blood research Score: 18

PMID: 42458108 | 10.1007/s44313-026-00153-2

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42458108/>

Open Access (Europe PMC/PMC): non rilevato

- **Tipo di studio/popolazione:** This systematic review evaluated the efficacy and safety of asciminib as first-line therapy in patients with newly diagnosed chronic-phase CML.
- **Risultato chiave:** Eligible studies included randomized controlled trials, prospective cohort studies, and compassionate use programs evaluating asciminib as first-line therapy in adult patients with chronic-phase CML.
- **Rilevanza clinica:** Treatment discontinuation due to adverse events was lower with asciminib (5.0%) than with second-generation TKIs (12.7%).

Mini-riassunto (da abstract, estrattivo):

Reattempt of tyrosine kinase inhibitor discontinuation after maintenance therapy with ponatinib in patients with chronic myeloid leukaemia in the chronic phase: Result of the Japan Adult Leukemia Study Group RE-STOP219 study.

Altro

British journal of haematology

Score: 14

PMID: 42422985 | 10.1111/bjh.70678

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42422985/>

Open Access (Europe PMC/PMC): non rilevato

- **Tipo di studio/popolazione:** Patients who had relapsed after prior tyrosine kinase inhibitor (TKI) discontinuation and subsequently regained molecular response 4.5 (MR4.5; defined as a BCR-ABL1 transcript level $\leq 0.0032\%$ on the International Scale [IS]) were enrolled, switched to ponatinib (15 mg/day) for 12 months and those who maintained MR4.5 discontinued TKI therapy.
- **Risultato chiave:** Of 50 enrolled patients, 49 received ponatinib and 41 proceeded to TKI discontinuation.
- **Rilevanza clinica:** In conclusion, ponatinib maintenance followed by a second attempt at TKI discontinuation may facilitate successful TFR in selected CML patients.

Mini-riassunto (da abstract, estrattivo):

Favorable tolerability of asciminib *versus* ATP-competitive tyrosine kinase inhibitors in the ASC4FIRST study of newly diagnosed patients with chronic-phase chronic myeloid leukemia.

Trial clinico

Haematologica

Score: 12

PMID: 42389818 | 10.3324/haematol.2025.300145

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42389818/>

Open Access (Europe PMC/PMC): non rilevato

- **Tipo di studio/popolazione:** Many patients with chronic myeloid leukemia (CML) treated with adenosine triphosphate (ATP)-competitive tyrosine kinase inhibitors (TKIs) experience persistent adverse events (AEs) that negatively impact daily living and the ability to remain on treatment.
- **Risultato chiave:** The phase 3 randomized ASC4FIRST trial established the overall favorable safety profile of asciminib in patients with newly diagnosed CML in chronic phase (CP).
- **Rilevanza clinica:** The safety and tolerability of asciminib observed in the ASC4FIRST trial demonstrate asciminib's excellent benefit-risk profile as a frontline therapy for a broad range of patients with newly diagnosed CML-CP.

Dose strategy / consolidamento (3)

Score: 0

Mini-riassunto (da abstract, estrattivo):

Therapeutic drug monitoring guides personalized ponatinib dose reduction for chronic myeloid leukemia.

Trial clinico

International journal of hematology

Score: 16

PMID: 42449088 | 10.1007/s12185-026-04254-7

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42449088/>

Open Access (Europe PMC/PMC): non rilevato

- **Tipo di studio/popolazione:** Although the phase II OPTIC trial showed that reducing the dose to 15 mg daily (QD) after achieving a response provided an optimal benefit-risk profile, this has not been adequately verified using plasma drug concentrations data.
- **Risultato chiave:** A 66-year-old woman with chronic-phase chronic myeloid leukemia (CP-CML) was switched to ponatinib because of nilotinib-induced grade 3 thrombocytopenia.
- **Rilevanza clinica:** A major molecular response (MMR) was achieved 30.6 months after starting ponatinib.

Mini-riassunto (da abstract, estrattivo):

Favorable tolerability of asciminib *versus* ATP-competitive tyrosine kinase inhibitors in the ASC4FIRST study of newly diagnosed patients with chronic-phase chronic myeloid leukemia.

Trial clinico

Haematologica

Score: 12

PMID: 42389818 | 10.3324/haematol.2025.300145

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42389818/>

Open Access (Europe PMC/PMC): non rilevato

- **Tipo di studio/popolazione:** Many patients with chronic myeloid leukemia (CML) treated with adenosine triphosphate (ATP)-competitive tyrosine kinase inhibitors (TKIs) experience persistent adverse events (AEs) that negatively impact daily living and the ability to remain on treatment.
- **Risultato chiave:** The phase 3 randomized ASC4FIRST trial established the overall favorable safety profile of asciminib in patients with newly diagnosed CML in chronic phase (CP).
- **Rilevanza clinica:** The safety and tolerability of asciminib observed in the ASC4FIRST trial demonstrate asciminib's excellent benefit-risk profile as a frontline therapy for a broad range of patients with newly diagnosed CML-CP.

Mini-riassunto (da abstract, estrattivo):

Pharmacokinetics of asciminib in a patient with chronic myeloid leukemia undergoing maintenance hemodialysis.

Altro

Cancer chemotherapy and pharmacology

Score: 9

PMID: 42412118 | 10.1007/s00280-026-04919-4

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42412118/>

Open Access (Europe PMC/PMC): non rilevato

- **Tipo di studio/popolazione:** A 72-year-old man with chronic phase chronic myeloid leukemia undergoing maintenance hemodialysis for chronic renal failure was treated with ASC.
- **Risultato chiave:** A major molecular response was achieved within 3 months without adverse events.
- **Rilevanza clinica:** ASC demonstrated negligible dialyzability, likely because of its high plasma protein binding (97.3%).

Sintesi (review/linee guida) (2)

Score: 0

Mini-riassunto (da abstract, estrattivo):

Asciminib as first-line therapy in chronic myeloid leukemia: a systematic review.

Linee guida / consenso

Blood research

Score: 18

PMID: 42458108 | 10.1007/s44313-026-00153-2

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42458108/>

Open Access (Europe PMC/PMC): non rilevato

- **Tipo di studio/popolazione:** This systematic review evaluated the efficacy and safety of asciminib as first-line therapy in patients with newly diagnosed chronic-phase CML.
- **Risultato chiave:** Eligible studies included randomized controlled trials, prospective cohort studies, and compassionate use programs evaluating asciminib as first-line therapy in adult patients with chronic-phase CML.
- **Rilevanza clinica:** Treatment discontinuation due to adverse events was lower with asciminib (5.0%) than with second-generation TKIs (12.7%).

Mini-riassunto (da abstract, estrattivo):

Financial Impact of Treatment Choice in Chronic Myeloid Leukemia: A Comparison of Later Generation TKIs versus Imatinib from Patient and Payer Perspectives.

Review ClinicoEconomics and outcomes research : CEOR Score: 5

PMID: 42445105 | 10.2147/CEOR.S626752

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42445105/>

Open Access (Europe PMC/PMC): non rilevato

- **Tipo di studio/popolazione:** The financial impact of later-generation TKIs (asciminib, bosutinib, dasatinib, nilotinib, and ponatinib) and imatinib was measured from patients' and payers' perspectives before (2024) and after the implementation of Inflation Reduction Act (2025), respectively.
- **Risultato chiave:** An all-payer blended US health plan with 1 million members could expect to have 200 patients with Philadelphia chromosome-positive CML eligible for TKI therapy.
- **Rilevanza clinica:** Both imatinib and later-generation TKI costs were projected to exceed annual OOP caps in 2025, likely resulting in minimal change in insured CML patients' OOP costs.

Resistenza / T315I / mutazioni (1)

Score: 0

Mini-riassunto (da abstract, estrattivo):

Ponatinib in CML and Ph+ ALL: balancing high efficacy with vascular safety.

Real-world / osservazionale Postgraduate medicine Score: 15

PMID: 42436599 | 10.1080/00325481.2026.2691687

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42436599/>

Open Access (Europe PMC/PMC): non rilevato

- **Tipo di studio/popolazione:** Ponatinib, a third-generation TKI, uniquely retains activity against the T315I 'gatekeeper' mutation and most other BCR:ABL1 variants, producing deep cytogenetic and molecular responses in resistant CML and improving survival in Ph+ ALL.
- **Risultato chiave:** Across the PACE and OPTIC programs, frontline Ph+ ALL combinations (hyper-CVAD- and blinatumomab-based), and real-world cohorts, ponatinib demonstrates robust antileukemic efficacy in diverse settings and after multiple prior TKIs.
- **Rilevanza clinica:** Clinically, response-based dosing-such as step-down from 45→30→15 mg in CML or 30→15 mg in Ph+ ALL upon molecular milestones-preserves efficacy while reducing exposure-related AOE risk, though events are not eliminated.

Real-world (utilità pratica) (1)

Score: 0

Mini-riassunto (da abstract, estrattivo):

Ponatinib in CML and Ph+ ALL: balancing high efficacy with vascular safety.

Real-world / osservazionale

Postgraduate medicine

Score: 15

PMID: 42436599 | 10.1080/00325481.2026.2691687

Link PubMed: <https://pubmed.ncbi.nlm.nih.gov/42436599/>

Open Access (Europe PMC/PMC): non rilevato

- **Tipo di studio/popolazione:** Ponatinib, a third-generation TKI, uniquely retains activity against the T315I 'gatekeeper' mutation and most other BCR:ABL1 variants, producing deep cytogenetic and molecular responses in resistant CML and improving survival in Ph+ ALL.
- **Risultato chiave:** Across the PACE and OPTIC programs, frontline Ph+ ALL combinations (hyper-CVAD- and blinatumomab-based), and real-world cohorts, ponatinib demonstrates robust antileukemic efficacy in diverse settings and after multiple prior TKIs.
- **Rilevanza clinica:** Clinically, response-based dosing-such as step-down from 45→30→15 mg in CML or 30→15 mg in Ph+ ALL upon molecular milestones-preserves efficacy while reducing exposure-related AOE risk, though events are not eliminated.